PRACTICE EXAM 28 / EXAM SET 04

Domain 7: Medical Law and Ethics

7 scored questions	4 answer choices per question	Answer key + explanations included

Instructions

Choose the single best answer (A, B, C, or D) for each question. Mark your responses before checking the answer key and detailed explanations at the end of this document. These practice questions are not official NHA examination items.

QUESTIONS

Q01 | Privacy Confidentiality and Information Release

> **SCENARIO** A patient submits a properly verified request for a copy of routine office notes and laboratory results from the patient's own record. The clinic is a HIPAA-covered entity, and no exception to the patient's access right applies. A billing employee asks the medical assistant to hold the request until an outstanding bill for medical care is paid. The assistant is responsible for routing access requests to the records team.

Which action correctly handles the unpaid medical bill in relation to this request?

A. Route the access request for normal processing without making payment of the medical-care bill a condition.

B. Hold the access request until the patient pays at least part of the medical-care bill.

C. Provide the laboratory results but withhold office notes until the medical-care bill is paid.

D. Require the treating clinician to authorize an exception to the patient's unpaid balance before processing access.

Q02 | Consent Patient Rights and Directives

> **SCENARIO** A patient has agreed to aspiration of a wrist cyst after a physician's explanation. Before the procedure begins, the physician proposes surgical excision instead. The patient tells the assistant, "I agreed to the needle procedure, but I have not decided about surgery." No emergency exists. Clinic policy requires the physician to discuss and obtain appropriate consent for the planned intervention before it proceeds.

Which response best addresses the change in the proposed procedure?

A. Treat the original agreement as consent because both procedures address the same cyst.

B. Ask the patient to initial the new procedure name without another physician discussion.

C. Alert the physician that consent for the revised intervention remains unresolved and pause preparation for it.

D. Explain the new procedure's risks independently and recommend whichever option appears simpler.

Q03 | Scope Ethics and Mandatory Reporting

SCENARIO A medical assistant realizes the next morning that an ordered dressing change performed the previous afternoon was not documented. The assistant has reliable information about the care personally provided. Clinic policy allows a clearly identified late entry using the actual entry date and time and the date and time of the care. The assistant is authorized to document this care but may not make a late entry appear contemporaneous.

Which documentation method preserves the timing and integrity of the record?

A. Enter the note using yesterday's date and time as though it had been written immediately after care.

B. Ask a coworker who was not present to document the care as an independent observation.

C. Omit the dressing change because a completed visit cannot contain additional documentation.

D. Add an authenticated late entry with the current entry time and the actual date and time of the dressing change.

Q04 | Consent Patient Rights and Directives

SCENARIO At a Medicare annual wellness visit, an older adult reports having no advance directive and says, "I do not want to complete a living will or choose a health care agent today. Will you cancel my visit?" The patient has decision-making capacity and no guardian. Clinic policy makes advance care planning optional, requires accurate documentation of whether an advance directive exists, and provides educational materials when patients want them.

Which response appropriately supports the patient's rights?

A. Ask a relative to complete the directive so the wellness visit can proceed.

B. Explain that planning is optional, document the patient's stated status accurately, and continue the visit.

C. Postpone the visit until the patient chooses someone to make future health care decisions.

D. Record that the patient has declined future life-sustaining treatment because no directive will be completed today.

Q05 | Scope Ethics and Mandatory Reporting

SCENARIO While working in a clinic covered by OSHA's Bloodborne Pathogens Standard, an assistant sustains a puncture from a used blood-contaminated needle and promptly washes the area. The clinic's exposure plan requires immediate incident reporting and access to confidential medical evaluation. A coworker says, "Finish the shift first; you will have to pay for any testing anyway." The source patient's infection status is not yet known.

Which action and understanding appropriately address the reported exposure?

A. Finish the shift and arrange evaluation only if the source patient's test later becomes positive.

B. Arrange testing through personal insurance because the exposure happened during paid work time.

C. Report the injury at the next staff meeting if symptoms develop before then.

D. Report the exposure immediately and use the employer-arranged confidential evaluation and follow-up at no cost to the employee.

Q06 | Privacy Confidentiality and Information Release

SCENARIO A patient previously authorized the clinic to send records to a disability-benefits administrator. One packet was sent last week in reliance on the valid authorization. Before a second packet is sent, the clinic receives and verifies the patient's written revocation. No other disclosure basis or exception permits this pending release. The assistant processes authorization updates under the clinic's release procedure.

What effect should the revocation have on the two releases?

A. Both releases become unauthorized, so the clinic must represent last week's permitted disclosure as unlawful.

B. Both releases remain permitted because signing the original authorization made it effective until its expiration date.

C. Stop the pending release under the revoked authorization; the revocation does not undo the earlier disclosure made in reliance on it.

D. Send the pending packet because revocation takes effect only when the benefits administrator agrees to it.

Q07 | Scope Ethics and Mandatory Reporting

SCENARIO Two patients have comparable education needs and the same scheduled visit length. A coworker proposes giving substantially less teaching time to the unemployed patient, stating, "That patient probably will not follow the plan anyway." There is no clinical or patient-preference reason for the difference. Clinic policy requires equitable care based on individual needs rather than assumptions about social status.

Which ethical principle most directly supports providing equitable teaching opportunities in this situation?

A. Autonomy, which supports a patient's voluntary choices about recommended care.

B. Justice, which supports fair treatment and equitable access to care.

C. Fidelity, which supports keeping professional commitments and promises.

D. Veracity, which supports communicating truthfully with patients.

ANSWER KEY & EXPLANATIONS

Practice Exam 28 | Domain 7: Medical Law and Ethics | 7 scored questions

01 A	02 C	03 D	04 B	05 D	06 C
07 B					

DETAILED EXPLANATIONS

Q01 | Correct answer: A

Route the access request for normal processing without making payment of the medical-care bill a condition.

A HIPAA-covered provider may not deny a patient's access to the patient's own records because the bill for health care services is unpaid. The assistant should route the verified request through the usual access process. Permissible copying fees are a separate issue and do not justify using the medical-care balance as a condition for access.

Q02 | Correct answer: C

Alert the physician that consent for the revised intervention remains unresolved and pause preparation for it.

Agreement to the original intervention does not establish agreement to the proposed different procedure. The physician must discuss the revised intervention's risks, benefits, and alternatives and address the patient's decision before it proceeds. The assistant supports this process by communicating the unresolved consent rather than treating a form change or an independent explanation as sufficient.

Q03 | Correct answer: D

Add an authenticated late entry with the current entry time and the actual date and time of the dressing change.

A properly identified late entry distinguishes when the care occurred from when it was documented. Under the stated policy, the assistant may accurately record personally provided care without backdating the entry or asking an uninvolved coworker to attest to it. Preserving the actual documentation time maintains an accountable record history.

Q04 | Correct answer: B

Explain that planning is optional, document the patient's stated status accurately, and continue the visit.

Advance care planning is an optional element of the annual wellness visit, and the stated clinic policy does not condition care on completing a directive. The assistant should respect the patient's choice and document the absence of a directive accurately. Declining to complete documents today neither authorizes a relative to decide for this capable patient nor establishes a refusal of future life-sustaining treatment.

Q05 | Correct answer: D

Report the exposure immediately and use the employer-arranged confidential evaluation and follow-up at no cost to the employee.

The clinic's plan requires prompt reporting of this occupational blood exposure. OSHA requires the employer to make confidential post-exposure evaluation and follow-up immediately available after an exposure report and at no cost to the employee. Unknown source status does not justify delaying evaluation until the shift ends, symptoms develop, or a positive test returns.

Q06 | Correct answer: C

Stop the pending release under the revoked authorization; the revocation does not undo the earlier disclosure made in reliance on it.

Once the clinic receives the verified written revocation, the authorization no longer supports the pending disclosure under the facts given. Revocation does not retroactively undo an action already taken in reliance on a valid authorization. The recipient's agreement is not needed for the clinic to honor the revocation, and the original expiration date does not prevent an earlier revocation.

Q07 | Correct answer: B

Justice, which supports fair treatment and equitable access to care.

Justice most directly concerns fairness in how care opportunities are provided. Reducing teaching solely because of an unsupported assumption about employment status conflicts with equitable, individualized care. Autonomy, fidelity, and veracity remain relevant professional principles, but the central issue here is unfair allocation of comparable care opportunities.

End of Practice Exam